Anastrozole · daily hormone therapy

Take anastrozole once daily continuously. There is no planned week off. The treatment setting determines the intended duration.

> Clinician supportive-care discussion draft. This is not an issued prescription.

Minimal emetogenic risk

No routine prophylactic antiemetic, corticosteroid premedication or granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) order is populated for this endocrine monotherapy. Review persistent nausea or other symptoms and prescribe individually if needed.

Medicine-specific support

- Tell the team if joint pain, hot flushes, vaginal dryness or sleep disturbance affects daily life. Bone thinning may cause no early symptoms, so keep the agreed bone-health reviews.
- No routine antiemetic, steroid premedication or G-CSF is required by this endocrine monotherapy. Ask for individual symptom support.
- Ask who books each check, when it is due and who reviews it. Tell the team if an expected appointment has not arrived.
- A functioning premenopausal ovarian state is not compatible with aromatase inhibitor (AI) monotherapy. Severe hepatic impairment is not recommended by NCCP; moderate impairment requires caution. Confirm menopausal status rather than assuming that absent periods after chemotherapy mean permanent menopause. Any ovarian-suppression treatment must have its own prescribed schedule.
- NCCP no renal adjustment including haemodialysis; severe renal impairment requires caution. No mild/moderate hepatic reduction; linked SmPC cautions moderate/severe hepatic dysfunction. NCCP severe hepatic disease not recommended: withhold pending review rather than invent a reduction.

Individual plan

Prescriber / date: _____ Pharmacy review: _____

Verify allergies, interactions, relevant organ function, treatment setting and the exact medicine before prescribing.

NCCP 00254 v5 · Monitoring and treatment map